



MC-2459



NABL CERTIFICATE



QR CODE

Date	18/Jun/2026 09:10AM	Report Date	18/Jun/2026 02:36PM
Name	Mrs. SANGEETA	Age/Sex	49 YRS/FEMALE
Lab No.	012606180027 (TILAK NAGAR)	UHID NO.	9666808
Panel Name	Standard	Slide No.	
Ref. Dr.	Dr. A.PRASAD	Sample Collection Date	18-Jun-2026 12:18PM
Report Status	Final Report	Barcode No.	01413015

CLINICAL PATHOLOGY

Test Name	Value	Unit	Bio. Reference Interval
AUTOMATED, ROUTINE URINE EXAMINATION,Urine			
PHYSICAL EXAMINATION			
COLOUR Visual	PALE YELLOW		PALE YELLOW
TRANSPARENCY Visual	CLEAR		CLEAR
CHEMICAL EXAMINATION			
SPECIFIC GRAVITY Ionic concentration	1.005		1.001-1.035
pH Double indicator	5.5		5.0- 9.0
URINE PROTEIN Protein error method	NEGATIVE		NEGATIVE
KETONES Legal test /Rothera Test	NEGATIVE		Negative
GLUCOSE glucose oxidase peroxidase	NEGATIVE		NEGATIVE
LEUCOCYTE ESTERASE Indoxyl reaction	NEGATIVE		NEGATIVE
NITRITE Greiss method	NEGATIVE		NEGATIVE
BLOOD Peroxidase reaction	ABSENT		ABSENT
UROBILINOGEN Diazonium salt	NEGATIVE		NEGATIVE
BILIRUBIN	NEGATIVE		
MICROSCOPIC EXAMINATION			
RED BLOOD CELL Flow Micro Imaging	NIL	/HPF	0-2
WHITE BLOOD CELL Flow Micro Imaging	1-2	/HPF	0-5
WHITE BLOOD CELL CLUSTER Flow Micro Imaging	ABSENT	/HPF	ABSENT
SQUAMOUS EPITHELIAL CELL Flow Micro Imaging	4-6	/HPF	0-5
NON-SQUAMOUS EPITHELIAL CELL Flow Micro Imaging	NIL	/HPF	0-1
HYALINE CASTS Flow Micro Imaging	NIL	/LPF	0-2
GRANULAR CASTS Flow Micro Imaging	NIL	/LPF	0-1
RBC CASTS Flow Micro Imaging	NIL	/LPF	0-1

Tanuja

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Dr. Deepa Khurana
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FATTY CASTS Flow Micro Imaging	NIL	/LPF	0-1
Other Casts	NIL		
AMORPHOUS URATES CRYSTALS Flow Micro Imaging	NIL	/HPF	NIL
AMORPHOUS PHOSPHATE CRYSTALS Flow Micro Imaging	NIL	/HPF	NIL
CALCIUM OXALATE DIHYDRATE CRYSTALS Flow Micro Imaging	NIL	/HPF	NIL
TRIPLE PHOSPHATE CRYSTALS Flow Micro Imaging	NIL	/HPF	NIL
URIC ACID CRYSTALS Flow Micro Imaging	NIL	/HPF	NIL
CALCIUM PHOSPHATE CRYSTALS Flow Micro Imaging	NIL	/HPF	NIL
CYSTINE CRYSTALS Flow Micro Imaging	NIL	/HPF	NIL
LEUCINE CRYSTALS Flow Micro Imaging	NIL	/HPF	NIL
UNCLASSIFIED CRYSTAL Flow Micro Imaging	NIL	/HPF	NIL
BACTERIA Flow Micro Imaging	NIL	/HPF	NIL
YEAST CELL Flow Micro Imaging	NIL	/HPF	NIL
SPERM Flow Micro Imaging	NIL	/HPF	NIL
MUCUS Flow Micro Imaging	ABSENT	/HPF	NIL

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Test Name	Value	Unit	Bio. Reference Interval
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CLINICAL BIOCHEMISTRY

Hb A1C, GLYCOSYLATED Hb ,EDTA Blood			
Hb A1C	5.50	%	4.0-5.60
HPLC			
ESTIMATED AVERAGE BLOOD GLUCOSE (eAG)	111.15	mg/dL	

Interpretation as per American Diabetes Association (ADA) 2025 Guidelines

Reference group	Non diabetic adults >18 years	At risk (prediabetes)	Diagnosing diabetes	Therapeutic goals for glycemic control
HbA1C	4-5.6%	5.7-6.4%	>= 6.5%	< 7%

1. FACTORS THAT INTERFERE WITH HbA1C MEASUREMENT:

Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements.

2. FACTORS THAT AFFECT INTERPRETATION OF HbA1C RESULTS:

Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g. recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and/or HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c.

Note : Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1c result does not correlate with the patient's blood glucose levels.

LDH, Serum

LDH	189.00	U/L	135-214
IFCC			

Reference Range:

Newborns (4-20 d) : 225-600 U/L
Children (2-15 y) : 120-300 U/L
Females : 135-214 U/L
Males : 135-225 U/L

The lactate dehydrogenase (LDH) enzyme is widely distributed in tissue, particularly in the heart, liver, muscles and kidneys. Elevated serum levels of LDH have been observed in a variety of disease states. The highest levels are seen in patients with megaloblastic anemia, disseminated carcinoma and shock. Moderate increases occur in muscular disorders, nephrotic syndrome and cirrhosis. Mild increases in LDH activity have been reported in cases of myocardial or pulmonary infarction, leukemia, hemolytic anemia and non-viral hepatitis.


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Test Name	Value	Unit	Bio. Reference Interval
LIVER FUNCTION TEST(LFT),Serum			
TOTAL PROTEIN Biuret Reaction	7.90	gm/dL	6.6-8.7
ALBUMIN,SERUM Bromocresol Green	4.90	gm/dL	3.5-5.2
GLOBULIN Calculated	3.00	gm/dL	2.3-3.5
A/G Ratio Calculated	1.63		1.1-2.5
SGOT (AST) IFCC with pyridoxyl phosphate	23.50	U/L	0-35
SGPT (ALT) IIFCC with pyridoxyl phosphate	24.80	U/L	0-35
ALKALINE PHOSPHATASE IFCC using p-NPP	115.00	U/L	40-129
GAMMA G.T. IFCC Szasz reaction	20.60	IU/L	6.0-42.0
TOTAL BILIRUBIN Diazo Method	0.39	mg/dL	<1.2
CONJUGATED BILIRUBIN (DIRECT) Diazo Method	0.18	mg/dL	0.0-<0.3
UNCONJUGATED BILIRUBIN(INDIRECT) Calculated	0.21	mg/dL	0.10-1.0

Interpretation:

It is a group of tests that are performed together to detect, evaluate, and monitor liver disease or damage. The panel usually consists of several tests that are run at the same time on a blood sample. These typically include:

Alanine Aminotransferase (ALT)

An enzyme found mainly in liver tissue and to a lesser extent in the heart, kidney and skeletal muscle. It's measurement is clinically useful in the diagnosis of liver and biliary disease.

Aspartate aminotransferase (AST)

An enzyme found in several parts of the body, including the heart, liver, and muscles. Since AST levels aren't as specific for liver damage as ALT, it's usually measured together with ALT to check for liver problems. When the liver is damaged, AST can be released into the bloodstream. A high result on an AST test might indicate a problem with the liver or muscles.

Gamma Glutamyl Transpeptidase (GGTP)

An enzyme that is found in many organs throughout the body, with the highest concentrations found in the liver. GGT is elevated in the blood in most diseases that cause damage to the liver or bile ducts.

Bilirubin, Total

It is used to detect an increased level of bilirubin in the blood. It helps to determine the cause of jaundice and diagnose conditions such as liver disease, hemolytic anemia, and blockage of the bile ducts.

Bilirubin, Direct

It measures a form of bilirubin that is conjugated (combined with another compound) in the liver; It is only increased in the case of liver disease.

Alkaline Phosphatase (ALP)

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An enzyme related to the bile ducts but also produced by the bones, intestines, and during pregnancy by the placenta (afterbirth). High levels of ALP may indicate liver inflammation, blockage of the bile ducts, or bone disease.

Albumin

Albumin is the main protein made by the liver. This test measures the level of albumin in the blood. A low result indicates that the liver isn't functioning properly.

Total Protein

The total protein test measures the total amount of protein in the blood and specifically looks for the amount of albumin and globulin. It also measures the ratio of albumin to globulin in the blood known as the A/G ratio.

Total Protein is useful in evaluating patients for nutritional status, liver disease, protein losing renal and gastrointestinal diseases.

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Test Name	Value	Unit	Bio. Reference Interval
KFT COMPLETE ,Serum			
UREA Urease	23.90	mg/dl	15.0-45.0
CREATININE,SERUM Jaffe Reaction	0.79	mg/dL	0.50-0.90
URIC ACID Uricase	4.88	mg/dL	2.4-5.7
TOTAL PROTEIN Biuret Reaction	7.90	gm/dL	6.6-8.7
ALBUMIN,SERUM Bromocresol Green	4.90	gm/dL	3.5-5.2
GLOBULIN Calculated	3.00	gm/dL	2.3-3.5
A/G Ratio Calculated	1.63		1.1-2.5
BLOOD UREA NITROGEN Calculated	11.17	mg/dL	6.0-20.0
SODIUM ISE Indirect	137.30	mmol/L	136-145
POTASSIUM ISE Indirect	4.11	mmol/L	3.5-5.1
CHLORIDE ISE Indirect	97.00	mmol/L	98-107
CALCIUM NM-BAPTA	10.10	mg/dl	8.6-10.0

Interpretation:

A kidney panel is a group of tests that may be performed together to evaluate kidney function. The tests measure levels of various substances, including

Electrolytes, electrically charged chemicals that are vital to normal body processes.

Electrolytes include

- Sodium
- Potassium
- Chloride

Minerals, electrically charged chemicals that are vital to normal body processes. Electrolytes include

- Phosphorus, vital for energy production, muscle and nerve function, and bone growth
- Calcium, essential for the proper functioning of muscles, nerves, and the heart and is required in blood clotting and the formation of bones.

Albumin

a protein essential to keeping fluid from leaking out of blood vessels and transporting hormones, vitamins, drugs, and ions like calcium throughout the body.

Waste products

- Urea, a nitrogen-containing waste product that forms from the metabolism of protein
- Creatinine, produced by the body's muscles. Almost all creatinine is eliminated by the kidneys.


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- Uric Acid, the end product of protein metabolism.

A:G Ratio, the calculated ratio of albumin to globulins. It may provide a clue as to the cause of the change in protein levels.


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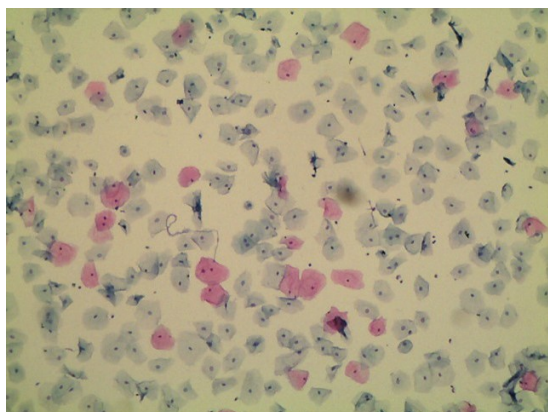
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Date	18/Jun/2026 09:10AM	Report Date	19/Jun/2026 06:21PM
Name	Mrs. SANGEETA	Age/Sex	49 YRS/FEMALE
Lab No.	012606180027 (TILAK NAGAR)	UHID NO.	9666808
Panel Name	Standard	Slide No.	0126-CY-3157
Ref. Dr.	Dr. A.PRASAD	Sample Collection Date	18-Jun-2026 12:19PM
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CYTOLOGY

LIQUID BASED CYTOLOGY***,LBC

THE 2014 BETHESDA SYSTEM FOR REPORTING CERVICAL/ VAULT CYTOLOGY



MICROSCOPY

Cervical smear shows admixture of intermediate and superficial squamous epithelial cells along with few parabasal cells. Background shows mild inflammation. No evidence of dysplasia or neoplasia seen in smears examined.

Specimen adequacy	Adequate
Endocervical cells	Absent
General categorization	NILM
Epithelial cells abnormality	Not seen
Reactive cellular changes	Absent
Inflammation	Mild
Organisms	Not seen
Atrophy	Absent
Other	NIL

INTERPRETATION:

NEGATIVE FOR INTRAEPITHELIAL LESION OR MALIGNANCY
NILM - MILD INFLAMMATORY CERVICITIS.

ADVICE:

Clinico-radiological correlation

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Advantages of LBC over Conventional Pap smear:

- Majority of cells collected are viewed and not discarded on sampler.
- Cells are viewed on a monolayer and show clearer morphology

Obscuring background elements like mucus, blood are not present, thereby improving the pick-up rate.

Disclaimer:

Gynaecological cytology is a screening procedure subject to both false negative and false positive results. It is most reliable when a satisfactory sample is obtained on a regular and repetitive basis. Results must be interpreted in context of the historic and current clinical information.

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ECHO, TMT, PFT & ECG

ECG

THE SYSTEM GENERATED GRAPH/IMAGE THAT YOU HAVE RECEIVED AFTER YOUR TEST IS THE FINAL REPORT. IF NOT RECEIVED YET PLEASE COLLECT IT FROM THE DISPATCH COUNTER.

COLOR DOPPLER ECHOCARDIOGRAPHY

OBSERVATIONS BY 2D AND M MODE EXAMINATION

Ed

IVS (mm)	11
LVID (mm)	41
LVPW (mm)	11
AO (mm)	25
LA (mm)	30
EF (%)	60 %

MITRAL VALVE	= Normal	AORTIC VALVE	= Normal
PULMONARY VALVE	= Normal	TRICUSPID VALVE	= Normal
PULMONARY ARTERY	= Normal		

2D STUDY OF REGIONAL WALL MOTION

There is no regional wall motion abnormality.

DOPPLER STUDY :-

VALVE	Vmax (m/s)	Pmax (mm Hg)	Pmean (mmHg)	Regurgitation
MITRAL	0.50			NIL
AORTIC	1.04	4.4	2.5	NIL
TRICUSPID				TRACE
PULMONARY	0.88			NIL

HR = 90 bpm,

Dr. Ashish Rawal
Consultant Physician &
Echocardiologist
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E/A = 0.60 DT = 222 ms E/e ' = 13.8

COLOR DOPPLER STUDY :-

No valvular stenosis . **Trace TR** . No intracardiac shunt.

SUMMARY AND COMMENTS :-

Normal sized Cardiac chambers with no chamber hypertrophy or dilatation.

No LV regional wall motion abnormality.

Normal LV systolic function . **Grade 1 LV diastolic dysfunction.**

No valvular stenosis . **Trace TR with gradient of 10 mmHg .**

There is no evidence of any thrombus, clot, vegetation or any other cardiac mass.

There is no pericardial pathology.

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HAEMATOTOLOGY

BLOOD GROUP ABO & Rh,EDTA Blood

ABO GROUP	O
RH FACTOR	NEGATIVE
Result Rechecked.	

METHODOLOGY: Column gel agglutination/Manual (Forward & Reverse)

LIMITATION:

- Exclusion of cold agglutinins must be done if any discrepancy between forward & reverse grouping is seen, by keeping the gel card prior to the test in incubator at 37° C for 15 min.
- Hemolysed, cloudy and aged samples should not be run as they can give false results.
- Antigen weakening is seen in group variants or patients with malignancy.
- Abnormal antibody expression is seen in patients with abnormal proteins & high potency antibodies.
- Improper plasma/serum may lead to improper results.

PT PROTHROMBIN TIME,Citrated Plasma

Mean Normal Prothrombin Time	11.70	Seconds	
Clot Detection			
Patient Value	11.1	Seconds	9.63-13.87
Clot Detection			
Prothrombin Ratio	0.95		
International Normalized Ratio (INR)	0.95		0.80-1.20
Calculated			

Interpretation:-

Prothrombin Time(PT) is used to monitor patients on anti -coagulant therapy as well as to diagnose coagulation disorders. Prothrombin Time (PT) assess as coagulation factors in extrinsic path way-(FVII) & common pathway- FX,FV, Prothrombin and fibrinogen.

Causes of prolonged PT (Prothrombin time)

- * Patients on anti -coagulant therapy
- * Liver disease
- * Vitamin K deficiency
- * DIC
- * Rarely a undiagnosed hereditary deficiency of coagulation factor - FVII,FX,FV,
- * Prothrombin & fibrinogen

INR (International normalized ratio)

USES

- * Standardises the interpretation in PT test result despite the variable testing method in different laboratories.
- * Used to monitor Patients on anti -coagulant therapy .
- * **Normal range of INR of healthy persons (not using warfarin) is 0.8 – 1.2 , target for those on warfarin is usually INR 2.0 to 3.0.**

Therapeutic target range of INR provides adequate anti coagulation for prevention of thrombosis and also checks excess dosage, which will cause bleeding .

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Limitations :

Preamalytical errors -Microclots in samples , partially filled tubes ,vacuum leaks, haemolysis, heparinised sample ,Hct > = 55 or < = 15, lipemic plasma , hyperbilirubinemia .

Note:- Result should be correlated clinically as prolonged PT neither predicts bleeding risk nor underlying diagnosis.

CBC WITH P/S AND ESR,EDTA Blood

HAEMOGLOBIN	13.1	gm/dl	11.5-15.0
Colorimetric			
PCV/HAEMATOCRIT	40.8	%	36-46
Pulse height			
RBC	4.54	mill/cumm	3.8-4.8
Sheath fluid impedance			
MCV	89.8	fl	83-101
Calculated			
MCH	28.9	Picogram	27-32
Calculated			
MCHC	32.1	gm/dl	31.5-34.5
Calculated			
RDW-CV	14.3	%	11.6-14.0
Calculated			
RDW SD	45.7	fL	39.0-46.0
Calculated			
Mentzer Index	20		
Calculated			
TOTAL LEUCOCYTE COUNT (TLC)	14330	/Cumm	4000-10000
SF cube cell analysis			
<u>DIFFERENTIAL LEUCOCYTE COUNT</u>			
NEUTROPHIL	65.7	%	40-80
Flowcytometry/Microscopy			
LYMPHOCYTES	19.5	%	20-40
Flowcytometry / Microscopy			
MONOCYTES	7.5	%	2-10
Flowcytometry / Microscopy			
EOSINOPHIL	7.0	%	1-6
Flowcytometry / Microscopy			
BASOPHILS	0.3	%	0-2
Flowcytometry / Microscopy			
IG	0.3	%	0.0-0.5
Flowcytometry			
ABSOLUTE NEUTROPHIL COUNT	9.41	1000/uI	2-7
Calculated			
ABSOLUTE LYMPHOCYTE COUNT	2.79	1000/uI	1-3
Calculated			
ABSOLUTE MONOCYTE COUNT	1.07	1000/uI	0.2-1.0
Calculated			
ABSOLUTE EOSINOPHIL COUNT	1.00	1000/uI	0.02-0.5
Calculated			

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MC-2459

NABL CERTIFICATE

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Date	18/Jun/2026 09:10AM	Report Date	18/Jun/2026 02:18PM
Name	Mrs. SANGEETA	Age/Sex	49 YRS/FEMALE
Lab No.	012606180027 (TILAK NAGAR)	UHID NO.	9666808
Panel Name	Standard	Slide No.	
Ref. Dr.	Dr. A.PRASAD	Sample Collection Date	18-Jun-2026 10:52AM
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ABSOLUTE BASOPHIL COUNT Calculated	0.04	1000/uI	0.02-0.1
PLATELET COUNT Sheath Fluid Impedance/Microscopy	3.12	Lakh/Cumm	1.5-4.1
PCT Calculated	0.4	%	0.19-0.40
PDW Calculated	16.6	fl	9.6-15.2
P-LCR Calculated	49.2	%	19.6-42.6
MPV Calculated	13.4	fl	9.2-12.1

PERIPHERAL SMEAR

RBCs are normocytic normochromic.
W.B.C. series shows neutrophilic leucocytosis and eosinophilia.
Platelets are adequate.
No haemoparasites in thin smear seen.

IMPRESSION: NEUTROPHILIC LEUCOCYTOSIS AND EOSINOPHILIA.

ADVICE: Clinical correlation

Note:

- Peripheral smear has been interpreted according to absolute values
- Smear prepared and stained with leishman stain for microscopy.

Note: If a complete blood count (CBC) indicates microcytic anemia and the Mentzer'S index is <13, Beta thalassemia trait is more likely. Conversely, a Mentzer index of >13 suggests higher likelihood of iron-deficiency anemia.

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Test Name	Value	Unit	Bio. Reference Interval
ESR	15	mm/hour	0-15

Capillary Photometry

ESR remains a widely used test for the screening and monitoring of infectious, autoimmune, malignant and other chronic inflammatory diseases and carries prognostic value .

Normal values of ESR by Westergren method

Age (Years)	Females (mm at 1st Hr)	Males (mm at 1st Hr)
< 50	< 12	< 10
51 - 60	< 19	< 12
61 - 70	< 20	< 14
70	< 35	< 30

Note :High ESR can be a incidental finding in asymptomatic patient. A mild to moderate elevated ESR without obvious underlying disease should prompt repeat testing after several months . However, an extremely high ESR even in asymptomatic patients should prompt the clinician to search for occult infections or inflammatory disease as stated above .

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Test Name	Value	Unit	Bio. Reference Interval
IMMUNOLOGY			
FT4 ECLIA	1.28	ng/dL	0.92-1.68

Comment:

Free T4 (fT4) is the unbound and biologically active form, which represents only 0.03 % of the total T4. The determination of fT4 has the advantage of being independent of changes in the concentrations and binding properties of these binding proteins; additional determination of a binding parameter (T-uptake, TBG) is therefore unnecessary. FT4 is a useful tool in clinical routine diagnostics for the assessment of the thyroid function. It should be measured together with TSH if thyroid disorders are suspected and is also suitable for monitoring treatment of secondary hypothyroidism (due to pituitary or hypothalamic disease) and of hyperthyroidism.

Interpretation:-

fT3	fT4	TSH	Interpretation
Normal	Normal	Normal	Euthyroid
Normal	Normal	Increased	Subclinical hypothyroidism
Decreased	Decreased	Increased	Primary hypothyroidism
Decreased/Normal	Decreased	Decreased	Secondary hypothyroidism
Normal	Normal	Decreased	Subclinical hyper thyroidism
Increased	Increased	Decreased	Primary hyper thyroidism
Increased	Normal	Decreased	T3 thyrotoxicosis

Pregnancy	fT3(pg/mL)	fT4(ng/dL)	TSH(uIU/mL)
1stTrimester	2.50-3.9	0.90-1.50	0.19-3.25
2ndTrimester	2.1-3.60	0.80-1.30	0.65-3.83
3rdTrimester	2.0-3.30	0.70-1.20	0.62-3.78

TSH (ULTRA SENSITIVE),Serum

TSH (ULTRA SENSITIVE) ECLIA	4.28	uIU/mL	0.27-4.20
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Interpretation:-

TSH measurements are particularly suitable for early detection or exclusion of disorders in the central regulating circuit between the hypothalamus, pituitary and thyroid. In case of suspected pituitary or hypothalamic disease, TSH levels are measured together with fT4 levels to confirm diagnosis.

Note: TSH levels are subject to circadian variation, rising several hours before the onset of sleep, reaching peak levels between 11 pm to 6 am. Nadir concentrations are observed during the afternoon. Diurnal variation in TSH level approximates +/- 50 %, hence time of the day has influence on the measured serum TSH concentration.

Clinical Use ·

- Diagnose Hypothyroidism and Hyperthyroidism and secondary thyrotoxicosis

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- Monitor T4 replacement or T4 suppressive therapy
- Quantify TSH levels in the subnormal range

Increased Levels: Primary hypothyroidism, Subclinical hypothyroidism, TSH dependent Hyperthyroidism, Thyroid hormone resistance **Decreased Levels:** Graves disease, Autonomous thyroid hormone secretion, TSH deficiency

Newborn

In a very low birth weight baby (particularly premature neonates) immaturity of the hypothalamic - pituitary - thyroid axis may mask primary congenital hypothyroidism. It is recommended that the test be repeated two weeks after birth in babies 1000 - 1500 gm and at four weeks in those < 1000 gm. Specimen collection prior to 24 hours of age, after blood transfusion and prematurity can affect this screening. A small number of children may test normal on the newborn screen but later develop hypothyroidism.

PREGNANCY Reference Range for TSH in uIU/mL (As per American thyroid Association)

1st Trimester	0.19 -3.25 uIU/mL
2nd Trimester	0.65-3.83 uIU/mL
3rd Trimester	0.62-3.78 uIU/mL

CA 125 (Cancer Marker), Serum

CA 125	23.30	U/mL	0-35
ECLIA			

Interpretation of Results

CA 125 is a useful tumor marker for evaluating therapy and monitoring disease status in patients under treatment for ovarian cancer. Post-operatively, the level of CA 125 correlates with tumor bulk and is a prognostic indicator of clinical outcome. It has been reported that patients with levels exceeding 35 U/mL have the highest risk for clinical recurrence. It has been reported in the literature that prior to a second-look laparotomy, a patient with levels of CA 125 greater than 35 U/mL is very likely to have tumor present at the surgery or to have a future recurrence. However, a level of CA 125 less than 35 U/mL prior to a second-look operation is not definitive evidence that the patient is free from residual tumor. Levels of CA 125 measured after a second-look operation provide strong indications of clinical outcome.

Patients with certain benign conditions, such as hepatic cirrhosis, acute pancreatitis, endometriosis, pelvic inflammatory disease, menstruation, and first trimester pregnancy, have shown elevated levels of CA 125. Elevated levels are found in 1 to 2% of healthy donors.

Elevated levels of CA125 may be associated with endometriosis , the first trimester of pregnancy, menstruation and cancer of the pancreas, stomach colon or rectum .

LIMITATIONS

For diagnostic purposes, the results obtained from this assay should always be used in combination with the clinical examination patient medical history, and other findings.

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Date	18/Jun/2026 09:10AM	Report Date	18/Jun/2026 01:47PM
Name	Mrs. SANGEETA	Age/Sex	49 YRS/FEMALE
Lab No.	012606180027 (TILAK NAGAR)	UHID NO.	9666808
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Ref. Dr.	Dr. A.PRASAD	Sample Collection Date	18-Jun-2026 10:52AM
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Test Name	Value	Unit	Bio. Reference Interval
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SEROLOGY

HBsAg, Hepatitis B surface Antigen, Serum

HBsAg (RAPID) NON-REACTIVE

Method: Immunochromatography

Interpretation:

Negative - Hepatitis B surface antigen not detected in the sample
Positive- Hepatitis B surface antigen detected in the sample

Comments:

This assay detects the first serological marker of Hepatitis B as early as 4-16 weeks after exposure. It persists during acute illness and disappears 12-20 weeks after onset of symptoms. The titers rise rapidly during the period of viral replication and is frequently associated with infectivity. Persistence of HbsAg for more than 6 months indicates development of carrier state or chronic liver disease.

Uses :

1. Routine screening of blood and blood products to prevent transmission of Hepatitis B virus (HBV) to recipients.
2. To diagnose suspected HBV infection and monitor the status of infected individuals
3. To evaluate the efficacy of antiviral drugs.
4. For Prenatal Screening of pregnant women.

Limitations :

1. This is only a screening test. All reactive samples should be confirmed by confirmatory tests.
2. Rarely false positive result can be obtained due to presence of other antigens or elevated levels of RF factor.
3. Additional follow up testing is required, if the test is non reactive with persisting clinical symptoms.

Anti HCV, Hepatitis C Antibody, CLIA, Serum

ANTI HCV 0.11 INDEX 0.0-1.0
CLIA

Method- Chemiluminescence Immunoassay (CLIA)

Interpretation-

>=1.00 indicates a reactive sample and the possible presence of anti HCV

<1.00 indicates a non- reactive sample, negative for anti- HCV

Comments-

The hepatitis C virus (HCV) is now known to be causative agent for most, if not all, blood borne non-A, non- B hepatitis (NANBH). Studies throughout the world indicate that HCV is transmitted through contaminated blood and blood products, through blood transfusions or through other close, personal contacts. The presence of anti- HCV indicates that an individual may have been infected with HCV and may be capable of transmitting HCV infection.

This test uses three recombinant hepatitis C virus encoded antigens- core, NS4, biotinylated NS3.

Limitations-

1. This is only a screening test and POSITIVE RESULTS SHOULD BE CONFIRMED BY SUPPLEMENTAL ASSAYS LIKE PCR.
Apart from that, for a definitive diagnosis, patients clinical history, symptomatology as well as serological

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- data should be considered.
- Patients with autoimmune liver diseases may show falsely reactive results.
 - A non-reactive result does not exclude the possibility of exposure to HCV. HCV antibodies may be undetectable in some stages of the infection and in some clinical conditions.
 - Heterophilic antibodies in the serum samples may cause interference in immunoassays. Specimens from patients receiving therapeutic doses of Biotin (Vitamin H, B7 or B8) may interfere causing a risk of false negative result.

HIV ANTIBODY 1 & 2 (WITH P24),Serum

HIV - 1/2 Antibodies	NON-REACTIVE
Immunochromatography (4th generation)	
P 24 antigen	NON-REACTIVE

Interpretation:

- Non Reactive: Antibodies to HIV 1 or II not detected in the sample
Reactive: Either P24 antigen or Antibodies to either HIV I or to HIV II are detected in the sample

Limitations:

- HIV Card Test is a 4th generation screening procedure. Reactive specimens should be retested with another method before final diagnosis. A non reactive result implies that either the patient has not been exposed to HIV or else the test has been carried out during the "window period" of the infection. Hence a non reactive result does not exclude the possibility of HIV infection.
- Some samples show cross reactivity for HIV antibodies. Following factors are found to cause false positive HIV antibody test results- Naturally occurring antibodies, passive immunization, leprosy, renal disorders, Tuberculosis, HSV, Hypergammaglobulinemia, Malignant neoplasms, Rheumatoid arthritis, Tetanus vaccination, Autoimmune diseases, Blood transfusion, Multiple myeloma, Haemophilia, Heat treated specimens, Lipemic serum, Anti nuclear antibodies, T-cell leukocyte antigen antibodies, EBV, HLA antibodies and other retroviruses.
- Advised post test counseling, available at City X Ray and Scan Clinic, Tilak Nagar centre.
- Samples found to be reactive by the above screening test must be confirmed by standard supplemental assays like Western Blot and HIV PCR.

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Date	18/Jun/2026 09:10AM	Report Date	18/Jun/2026 11:27AM
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Lab No.	012606180027 (TILAK NAGAR)	UHID NO.	9666808
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ULTRASOUND

USG BREAST

USG DONE ON 4D MACHINE

Scanning was done using a 13.0 MHz high resolution probe.

Scans show both breasts of homogenous echopattern.

A well circumscribed, oval shaped, wider than taller hypoechoic lesion measuring 4.6 x 10.5 mm is seen at 2 O' clock position of left breast in zone II - Benign - ? Fibroadenoma - BIRADS 3 (Advise - Short term interval follow up).

No nodule seen in right breast.

No calcification seen.

Skin thickness is normal.

No obvious ductal dilatation.

No obvious axillary lymphadenopathy seen on either side.

PLEASE CORRELATE CLINICALLY.


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Date	18/Jun/2026 09:10AM	Report Date	18/Jun/2026 12:41PM
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X-RAY DIGITAL

X RAY CHEST PA

- * Lung parenchyma and bronchovascular markings are normal.
- * Bilateral hilar shadows are normal.
- * Trachea is central.
- * Cardiothoracic ratio is within normal limits.
- * Bilateral cardiophrenic & costophrenic angles are clear.
- * Bony thoracic cage, extrapulmonary soft tissues and domes of diaphragm are normal.

Please correlate clinically.

*** End Of Report ***

*Disclaimer: This is an electronically validated report, if any discrepancy found should be confirmed by user.

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